

BUNDIBUGYO VIRUS DISEASE OUTBREAK

Democratic Republic of the Congo | Uganda | France

Weekly External Situation Report 16, Data as of 30 August 2026

Key Figures at a Glance (Democratic Republic of the Congo)

6 Provinces 60 Health Zones Affected	5 Provinces 50 Health Zones Active Transmission*	1 383 Cases Recovered	62.2% Bed Occupancy	24 330 Contacts under follow up
814 Active Cases in Isolation	160 Health Worker Cases	45 Health Worker Deaths	1 308 Bed Capacity	22 Testing Laboratories

Note: Health zones with active transmission have reported at least one case in the last 21 days.

Summary

Country	New Confirmed Cases (Last 7 days)	New Confirmed Deaths (Last 7 days)	Confirmed Cases (Total)	Confirmed Deaths (Total)	Probable Cases	Probable Deaths	Total Cases (Confirmed & Probable)	Total Deaths (Confirmed & Probable)	CFR (%) Confirmed & Probable	Outbreak Status
Democratic Republic of the Congo	516	270	6 100	2 950	-	-	6 100	2 950	48.4%	Active
Uganda	0	0	20	2	1	1	21	3	14.3%	Ended
France	0	0	1	0	0	0	1	0	0.0%	Ended

Note: Only confirmed and probable cases are reported.

Event description

Democratic Republic of the Congo

The Bundibugyo virus disease (BVD) outbreak in the Democratic Republic of the Congo continues to evolve, characterized by sustained transmission, high mortality and continued geographic expansion. The epidemiological picture is increasingly heterogeneous, with persistent transmission in established hotspots in Ituri, intensifying transmission in parts of Nord-Kivu and Haut-Uélé, and the continued emergence of cases in previously unaffected health zones. Since [External Situation Report #15](#), a further 516 confirmed cases and 270 confirmed deaths have been reported, bringing the cumulative total to 6100 confirmed cases, including 2950 deaths (CFR 48.4%), as of 30 August

2026. The number of affected health zones has increased from 57 to 60 across six provinces, with Biena and Manguredjipa in Nord-Kivu and Ganga in Bas-Uélé newly affected since the previous update. Ituri remains the epicentre, accounting for 82.2% of cumulative confirmed cases, although continued expansion in Nord-Kivu and Bas-Uélé underscores the increasingly dispersed geographic pattern of the outbreak.

During the most recent 21 days (10 – 30 August 2026), 1719 confirmed cases were reported nationally. Compared with 1958 cases during the preceding 21-day period (20 July – 9 August 2026), this represents a decrease of 239 cases (–12.2%). This decline should be interpreted cautiously given potential reporting delays and retrospective data reconciliation and does not, on its own, establish that transmission is declining. The national trend continues to mask substantial geographic heterogeneity. Reported cases declined by 21.6% in Ituri, from 1601 to 1255, but increased by 33.2% in Nord-Kivu, from 256 to 341, and by 12.4% in Haut-Uélé, from 97 to 109. Consequently, Ituri’s contribution to newly reported cases fell from 81.8% to 73.0%, while Nord-Kivu’s contribution increased from 13.1% to 19.8% and Haut-Uélé’s from 5.0% to 6.3%. A further 10 cases were reported in Tshopo and four in the newly affected Bas-Uélé during the latest period. The latest provincial cumulative totals confirm that Ituri remains the principal focus, but transmission is becoming progressively less concentrated in the original epicentre.

During the same period, 939 confirmed deaths were reported nationally, compared with 1044 deaths during the preceding 21 days, representing a decrease of 105 deaths (–10.1%). This decline was again driven predominantly by Ituri, where reported deaths decreased from 809 to 654 (–19.2%). In contrast, deaths increased from 197 to 228 (+15.7%) in Nord-Kivu and from 37 to 50 (+35.1%) in Haut-Uélé. Consequently, Ituri’s contribution to newly reported deaths declined from 77.5% to 69.6%, while Nord-Kivu’s increased from 18.9% to 24.3% and Haut-Uélé’s from 3.5% to 5.3%. Tshopo reported four deaths and Bas-Uélé three during the latest period. Taken together, the decline in both reported cases and deaths suggests a reduction in the nationally reported disease burden. However, reporting delays and retrospective data reconciliation limit interpretation, while simultaneous increases in cases and deaths in Nord-Kivu and Haut-Uélé demonstrate that transmission remains substantial and is continuing to redistribute geographically.

Figure 1. Daily trend in confirmed Bundibugyo virus disease cases, with seven-day moving average, by date of report, Democratic Republic of the Congo, as of 30 August 2026

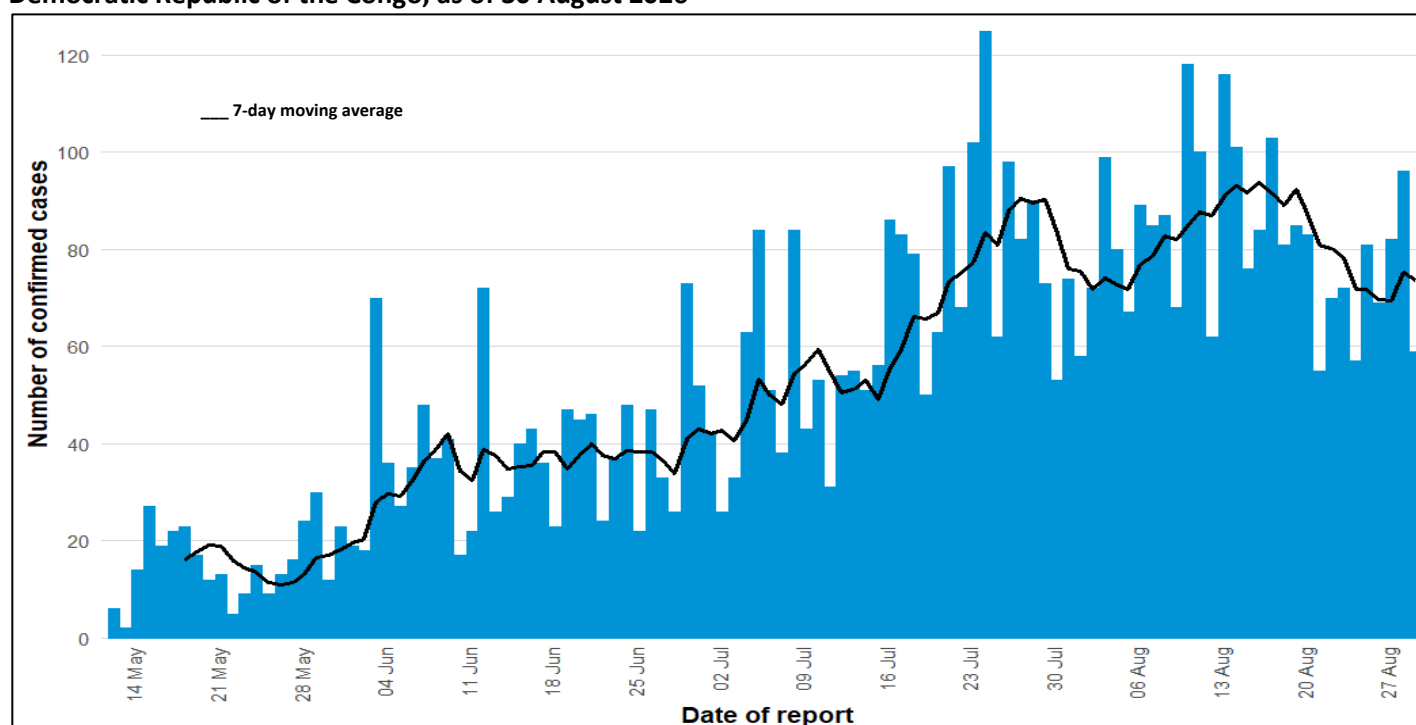


Table 1. Bundibugyo virus disease cases and deaths by province across consecutive 21-day periods, 20 Jul – 09 August 2026 compared with 10 – 30 August 2026, Democratic Republic of the Congo

Province	Cases			Deaths		
	Cases 20 Jul–9 Aug	Cases 10–30 Aug	Change	Deaths 20 Jul–9 Aug	Deaths 10–30 Aug	Change
Ituri	1 601	1 255	–21.6%	809	654	–19.2%
Nord-Kivu	256	341	+33.2%	197	228	+15.7%
Haut-Uélé	97	109	+12.4%	37	50	+35.1%
Tshopo	4	10	+150.0%	1	4	+300.0%
Sud-Kivu	0	0	—	0	0	—
Bas-Uélé	0	4	New	0	3	New
National	1 958	1 719	–12.2%	1 044	939	–10.1%

Notes: Data are as of 30 August 2026. Net change is the absolute difference between 10 – 30 August and 20 July – 09 August. Relative change is the net change expressed as a percentage of the preceding 21-day period. “New” indicates that no cases or deaths were reported during the preceding period, so a percentage change cannot be calculated.

Figure 2. Proportion of Bundibugyo virus disease cases and deaths by province across two consecutive 21-day periods, 20 Jul – 09 August 2026 compared with 10 – 30 August 2026, Democratic Republic of the Congo

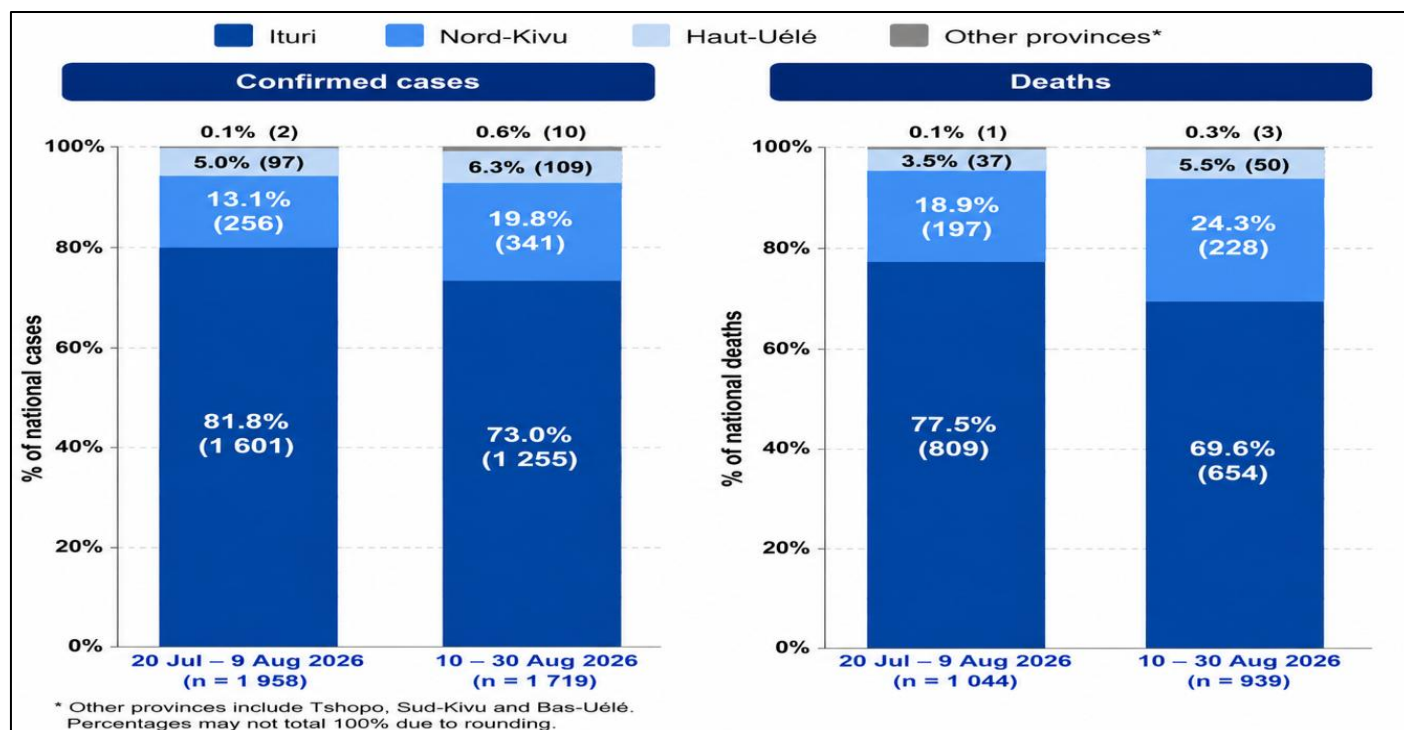
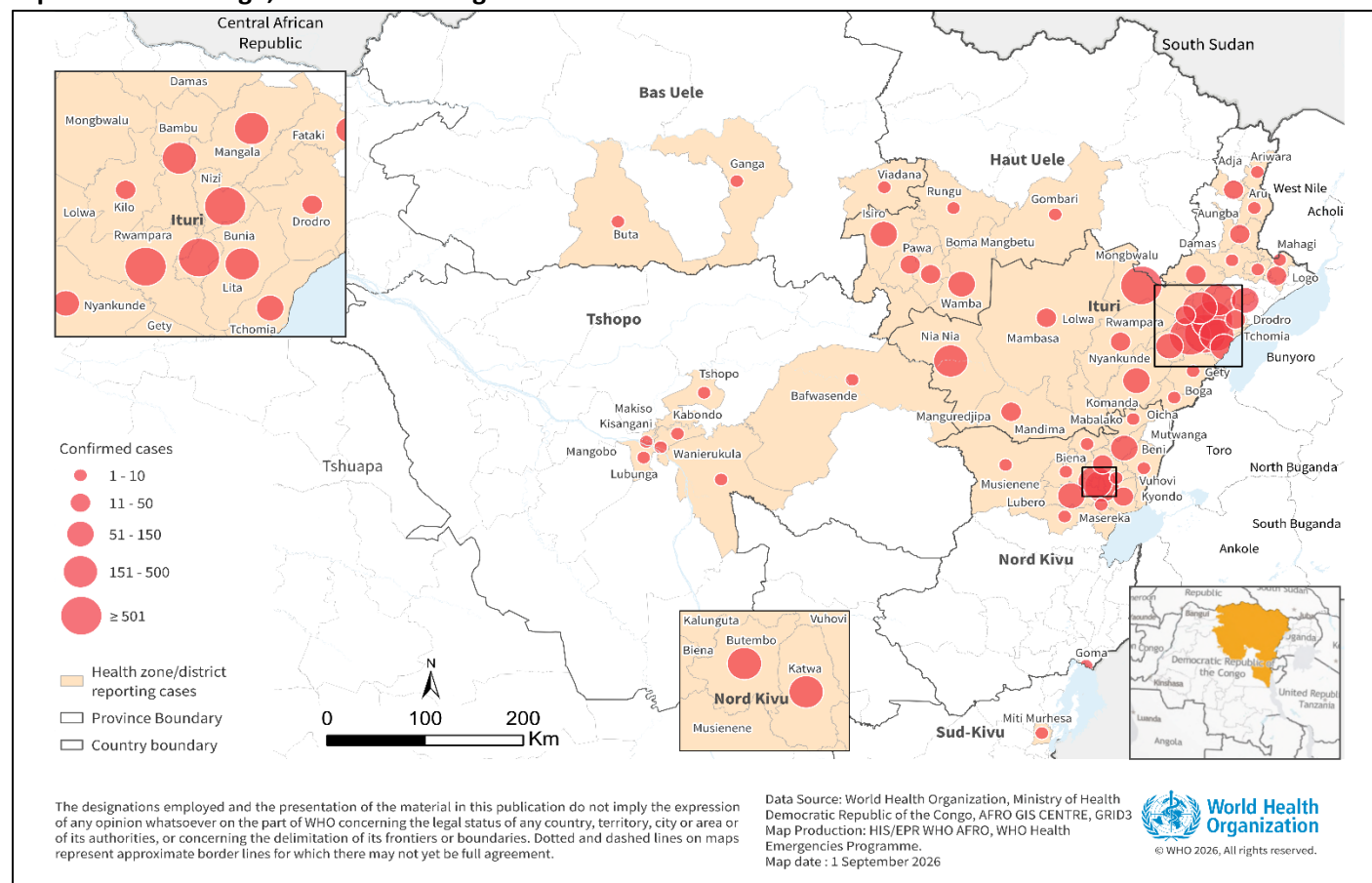


Figure 3. Geographical distribution of cumulative confirmed cases of Bundibugyo virus disease in the Democratic Republic of the Congo, data as of 30 August 2026



At health-zone level, the national decline masks substantial heterogeneity across established and emerging transmission foci. Of the 60 health zones affected since the start of the outbreak, comparison of the two consecutive 21-day periods shows that reported cases increased in 20 health zones and decreased in 22. A further 10 previously affected health zones reported no confirmed cases during the latest 21 days (Adja, Kambala, Logo and Mahagi in Ituri; Goma and Lubero in Nord-Kivu; Rungu in Haut-Uélé; Lubunga and Wanie-Rukula in Tshopo; and Miti-Murhesa in Sud-Kivu), while incidence was unchanged in one health zone (Bafwasende, Tshopo). Seven health zones were newly affected during the latest 21-day period.

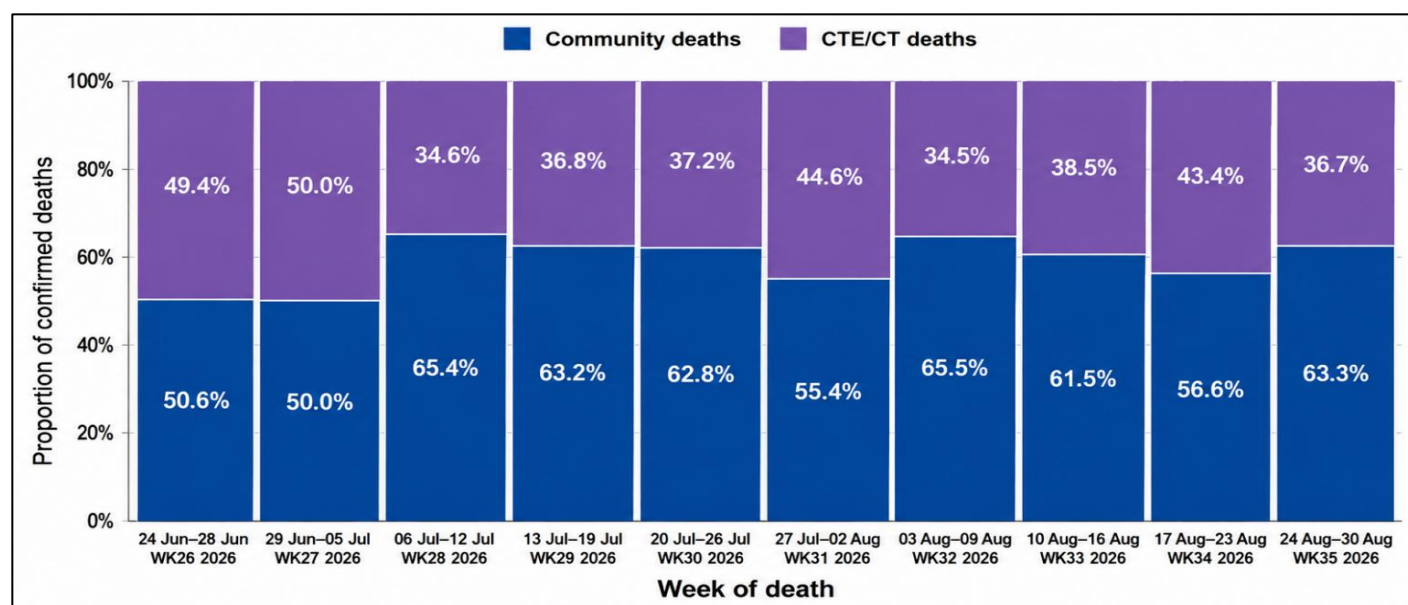
In Ituri, the reduction was driven particularly by Mongbwalu, where reported cases declined from 183 to 44 (−139; −76.0%), and Nizi, from 277 to 182 (−95; −34.3%). Cases also declined in Lita, from 84 to 61 (−23; −27.4%), and Rwampara, from 242 to 191 (−51; −21.1%). Bunia, however, remained the largest active focus and increased from 363 to 383 (+20; +5.5%). Other Ituri health zones also recorded increases, notably Mangala, from 53 to 126 (+73; +137.7%), Nia-Nia, from 61 to 80 (+19; +31.1%), and Bambu, from 31 to 66 (+35; +112.9%). Outside Ituri, transmission continued to intensify in Nord-Kivu, particularly in Katwa, where cases increased from 112 to 165 (+53; +47.3%), and Beni, from 30 to 66 (+36; +120.0%). In Haut-Uélé, Isiro increased from 18 to 51 (+33; +183.3%) and Wamba from 8 to 48 (+40; +500.0%). These patterns show that the overall decline in Ituri is being driven by substantial reductions in several established hotspots rather than a uniform decrease across the province; Bunia remains highly active, while other foci within Ituri and in Nord-Kivu and Haut-Uélé continue to expand.

Reported deaths show a similar geographic redistribution but with important differences from the case trends. In Ituri,

deaths declined substantially in several established hotspots, including Mongbwalu, Nizi, Lita and Rwampara, consistent with the reduction in reported cases in these areas. However, mortality remained substantial in Bunia, while increases in cases in Mangala, Nia-Nia and Bambu indicate continued transmission within the province despite the overall decline. In Nord-Kivu, increasing mortality accompanied the continued growth of the Katwa and Beni foci, while in Haut-Uélé the increase in deaths was concentrated particularly in Isiro and Wamba. Overall, the health-zone pattern reinforces the provincial analysis: transmission is declining in several formerly dominant Ituri hotspots but is not declining uniformly. Interpretation of health-zone mortality trends in Ituri should however remain cautious, particularly because cumulative death counts have undergone continuing reconciliation and redistribution across health zones, with 329 confirmed deaths in Ituri awaiting allocation to health zone as of 30 August 2026.

Mortality remains high, with 270 confirmed deaths reported during the last seven days, including 171 (63.3%) in the community and 99 (36.7%) in BVD treatment facilities. The persistently high proportion of community deaths (defined as both deaths at home and at non-BVD health facilities) highlights continued challenges with early detection, referral and timely access to treatment, while mortality in BVD treatment facilities may reflect late presentation, quality of care and patient vulnerabilities, including age, malnutrition and comorbidities; further analysis to understand the risk factors is underway.

Figure 4. Weekly proportion of confirmed Bundibugyo virus disease deaths by place of death, Democratic Republic of the Congo, epidemiological week 26 – 35, 2026



Uganda and France

Both Uganda and France have not reported new cases for more than 42 days. The outbreak in these countries is considered over by WHO's norm of 42 days without any cases.

Risk Assessment

According to WHO's latest Rapid Risk Assessment as of 20 August 2026, the risk of further spread remains very high within the Democratic Republic of the Congo and high for neighbouring countries sharing land borders with the country. The assessment reflects sustained transmission, continued geographic expansion, high mortality, population mobility, insecurity and persistent response challenges. The risk is considered low elsewhere in Africa and globally. The second IHR Emergency Committee, convened on 18 August 2026, also reviewed the evolving situation and emphasized that the outbreak remains far from controlled, and continues to constitute a Public Health Emergency of International Concern.

Public health response

Coordination

- The WHO–Africa CDC Continental Incident Management Support Team (IMST) in Kampala continued to coordinate implementation of the Joint Continental Preparedness and Response Plan under the “One Response” approach. Covering June – November 2026, the plan guides operational coordination, information sharing, partner engagement and preparedness support across affected and high-risk countries.
- In the Democratic Republic of the Congo, coordination continued to adapt to the outbreak’s expanding geographic footprint, with greater decentralization of response operations and national and provincial support to priority areas in Nord-Kivu and Haut-Uélé, including along key transmission and population-mobility corridors.
- WHO, Africa CDC and partners continued to align technical assistance, deployments and operational resources with evolving transmission patterns and response gaps. UNICEF, International Organization for Migration (IOM), Médecins Sans Frontières (MSF), Alliance for International Medical Action (ALIMA), International Medical Corps (IMC), Samaritan’s Purse, the European Union, Global Outbreak Alert and Response Network (GOARN), Emergency Medical Teams, StandBy partners and others supported field operations, laboratory activities, logistics and expert deployments in affected and at-risk areas.

Operations Support and Logistics (OSL)

- The emergency supply pipeline increased to US\$11.59 million, up from US\$11.38 million in the previous update. Supplies worth US\$5.11 million (44%) have been delivered, US\$5.57 million (48%) are in transit and approximately US\$0.90 million (8%) are not yet ready for shipment.
- Delivery of supplies continued to progress, with cumulative delivered cargo reaching 346.7 metric tonnes and 1 971.5m³, compared with 332.3 metric tonnes and 1 904.4m³ in the previous update.
- Critical commodities continued to reach the Ituri and Kivu hubs. Cumulative deliveries include 78 274 hooded coveralls, 5.53 million examination gloves, 158 800 Level 3 gowns, 261 hospital beds, 10 076 infrared thermometers, 20 016 RadiOne PCR tests and 7200 manual PCR tests. A further 174 200 hooded coveralls, 4.19 million examination gloves and 267 800 Level 3 gowns are in transit.
- Personal protective equipment (PPE) and infection prevention and control (IPC) supplies remain the largest procurement category at US\$5.17 million, followed by medical equipment (US\$1.95 million), laboratory supplies (US\$1.87 million), logistics (US\$1.29 million) and medicines (US\$0.89 million).
- Pipeline readiness improved, with 92% of supplies by value either delivered or in transit, leaving only 8% not yet ready for shipment. International suppliers remain the predominant source, accounting for approximately 81% (US\$9.37 million) of procurement value, underscoring the importance of maintaining timely international shipment and onward distribution.

Surveillance

- Surveillance activities continued across affected and at-risk areas, including active case finding, alert management and investigation, contact tracing, mortality surveillance, community-based surveillance, and surveillance at points of entry (PoEs) and points of control (PoCs).
- On 30 August, 1628 alerts were recorded across five affected provinces, of which 1541 (94.7%) were verified. All 258 validated suspected cases were investigated (100%), and 133 were transferred to treatment centres.
- Contact-tracing workload remained high, with 24 330 contacts under follow-up, of whom 21 002 (86.3%) were seen during the preceding 24 hours. Follow-up was highest in Tshopo (100%), followed by Ituri (89.4%) and Nord-Kivu (84.8%), but remained substantially lower in Haut-Uélé (68.5%) and Bas-Uélé (63.3%). The continued low performance in Haut-Uélé and Bas-Uélé was attributed to insufficient dedicated contact-tracing teams.
- At PoEs and PoCs, 254 483 travellers were recorded on 30 August, with 98.2% screened. Ten alerts were detected, including six subsequently validated as suspected cases and transferred for care. Reporting completeness across PoEs/PoCs was 83.7%, with important gaps in Haut-Uélé (36%), Bas-Uélé (55%) and Sud-Kivu (55.5%).

Laboratory

- Diagnostic capacity continued to expand and decentralize, increasing from two laboratories at outbreak onset to 22 active laboratories, supported by 42 RadiOne POC PCR and 9 RT-PCR platforms, with capacity exceeding 3036 samples per day. New laboratory capacity has been established in Buta, Butembo and Durba, with further platforms in the pipeline.
- Laboratory workforce and regional preparedness are being strengthened, including training of approximately 60 personnel/trainers from the Democratic Republic of the Congo and Uganda in field biosecurity and specimen management, with cascade training planned for other countries in the region.
- Priorities are now to consolidate and sustain the expanded network through implementation of the updated decentralization plan; reliable supplies, equipment, training and supervision; and improved turnaround-time data capture, specimen management and partner reporting. WHO is procuring approximately US\$1.5 million in laboratory supplies and critical equipment to support new and existing laboratories.

Case Management

- Demand for clinical care remained high as transmission continued and expanded geographically. As of 30 August 2026, 814 patients were in isolation or receiving care nationally. A total of 49 treatment facilities are active, while available bed capacity has increased to 1308.
- Localized treatment and referral gaps persisted despite overall bed availability, with saturation in parts of Ituri, full occupancy of suspected-case beds in Nord-Kivu, limited treatment capacity in Haut-Uélé and Tshopo, and insufficient ambulance capacity across several affected provinces. These constraints are increasingly important as transmission expands into areas with weaker treatment infrastructure.
- Cumulative recoveries increased to 1383 as of 30 August 2026.

Research and Development

- On 17 August 2026, the International Coordinating Group on Vaccine Provision allocated 70 000 doses of ERVEBO, at the request of the Government of the Democratic Republic of the Congo. ERVEBO is licensed for the prevention of Ebola virus disease caused by *Zaire ebolavirus*. The allocation includes 20 000 doses for a Phase 3 clinical trial to evaluate the vaccine's effectiveness against Bundibugyo virus disease and 50 000 doses for frontline and healthcare workers, in line with the current recommendations from the WHO Strategic Advisory Group of Experts on Immunization (SAGE).
- Clinical research on therapeutics continued to expand in the Democratic Republic of the Congo. The WHO-sponsored PARTNERS platform trial is evaluating MBP134, remdesivir and their combination for the treatment of confirmed BVD, with enrolment continuing across five study sites. The trial is coordinated by the National Institute for Biomedical Research (INRB), the Institute of Tropical Medicine Antwerp and the University of Oxford, with WHO sponsorship and support from Africa CDC and other partners.
- WHO's therapeutics advisory group has prioritized obeldesivir for clinical evaluation as post-exposure prophylaxis. The EBO-PEP study continued to evaluate oral obeldesivir as post-exposure prophylaxis for high-risk contacts and the administration of remdesivir in children, as well as in pregnant and breastfeeding women who have had high-risk exposure, with efforts underway to expand enrolment through more flexible delivery approaches.
- The MBOTE natural history study has commenced, expanding evidence generation on the clinical presentation, progression and outcomes of BVD and complementing ongoing interventional therapeutic studies.
- BDBV vaccine development progressed from trial initiation to vaccination of trial participants. The University of Oxford's Phase I BD-Ebov trial of the ChAdOx1 BDBV vaccine is evaluating safety and immune responses in 50 healthy adults aged 18 – 55 years. The first participant was vaccinated on 24 July 2026, and recruitment and vaccination are continuing. Approximately 620 000 doses have been manufactured and stockpiled by the Serum Institute of India, including 4000 investigational doses supplied for clinical evaluation. Preparations for additional studies in Uganda are continuing, subject to regulatory approval. Phase 1 trials for Moderna's candidate BDBV vaccine, mRNA-1469, began in Canada on 4 August 2026.

Infection Prevention and Control (IPC) and Water, Sanitation and Hygiene (WASH)

- IPC capacity continues to expand, including IPC ring interventions, decontamination, supportive supervision and supplies; however, implementation remains uneven in areas of intense or expanding transmission.
- Safe and dignified burial capacity is increasing through additional teams, training and deployment, but community resistance, insecurity and access constraints continue to limit timely SDB coverage.
- Healthcare worker infections remain a major concern, with 160 confirmed infections and 45 deaths, underscoring persistent healthcare-associated transmission risks and the need to strengthen triage, PPE use and adherence to IPC standards.

Risk Communication and Community Engagement (RCCE)

- RCCE activities continued to expand across affected provinces, supporting early case detection, contact tracing, timely care-seeking, safe and dignified burials, and acceptance of response measures.
- Engagement of high-risk and mobile populations increased, including outreach to motorcycle-taxi associations in Nord-Kivu and targeted household and community activities in affected areas.
- RCCE expanded into newly affected areas, including Tshopo, through household engagement, local team training, radio programmes and community sensitization. An increasing number of community health workers are being mobilised and integrated into the response.
- Community feedback and trusted local networks continued to guide response activities, helping to address misinformation and improve acceptance of surveillance, referral and burial interventions. Feedback is reported on daily basis and prioritised for interventions. The second round of community rapid assessment is finalised and analysis is being shared.
- Mistrust and resistance remain important challenges, including refusals of safe burials, post-mortem sampling and decontamination, underscoring the need for sustained trust-building and locally adapted engagement.

Preventing and Responding to Sexual Exploitation, Abuse, and Harassment (PRSEAH)

- PRSEAH coordination is being strengthened at global, national and operational levels, including finalization of the national and inter-agency PRSEAH response plan and mapping of technical capacity across the Democratic Republic of the Congo and high-risk areas.
- Community awareness and prevention activities are expanding, with 962 community members reached and 396 people briefed across Ituri, South Kivu and Tshopo, alongside strengthened capacity of community mobilizers.
- Survivor-centred support and safeguarding mechanisms are being reinforced, including mapping of Gender-Based Violence and victim-support services, identification of service gaps, and strengthening of community-based complaints and reporting mechanisms.
- Priority gaps remain in geographic coverage and partner alignment, with efforts focused on extending PRSEAH capacity across affected health zones, strengthening community and media outreach, and training community health workers and PRSEAH focal points, including in Aru and Mambasa.

Public health preparedness

Operational Readiness

- Regional cross-border coordination continued to advance, supported by a high-level meeting co-convened by WHO, Africa CDC, the Economic and Monetary Community of Central Africa (CEMAC) and the Central African Republic in Bangui from 14 – 15 August 2026. Ministers and delegations from the Democratic Republic of the Congo, Central African Republic, Republic of the Congo, South Sudan and Uganda mapped high-risk cross-border corridors and conducted field visits to the PK24 treatment centre, Mongoumba border port, laboratory and

community health centre. Engagement with the President of the Central African Republic on 17 August reinforced the need to shift from reactive response to sustained preparedness. Countries agreed to finalize a multi-country Memorandum of Understanding, develop harmonized workplans and address funding gaps.

- Regional preparedness continued to improve, with 31 WHO African Region Member States completing and submitting readiness assessments, increasing regional coverage to 66%.
- Nine high-priority countries completed repeat readiness assessments, reflecting continued efforts to strengthen national preparedness capacities: Angola, Central African Republic, South Sudan, Tanzania, Kenya, Republic of the Congo, Rwanda, Uganda and Zambia.
- Regional technical deployments continued to support preparedness and response capacity in affected and high-risk Member States, including Burundi, Central African Republic, Democratic Republic of the Congo and South Sudan.

Border Health, Travel and Mass Gatherings

- Screening and public health measures have been reinforced at airports, high-risk border crossings, major ports and internal transit routes in the Democratic Republic of the Congo and Uganda. National authorities, IOM, and partners continue to support the implementation of WHO border health guidance, strengthen cross-border coordination, and facilitate the timely detection, reporting and referral of suspected cases at PoEs and PoCs along priority travel corridors.
- Cross-border preparedness and response are being strengthened through enhanced coordination among the Democratic Republic of the Congo, Uganda, the Central African Republic and South Sudan to improve the early detection of cases and reduce the risk of cross-border spread. Priority actions include strengthening coordinated cross-border disease surveillance, event-based surveillance, active case finding, joint investigations and contact tracing where required, and real-time information sharing through regular cross-border coordination meetings and harmonized reporting mechanisms.
- Priority operational actions include rapid assessments and mapping of PoEs, strengthening surveillance along high-risk cross-border mobility corridors, including informal crossings, and major congregation points, and ensuring readiness capacities through the provision of trained personnel, equipment, PPE, referral mechanisms, disinfectants, risk communication materials and contingency plans for the safe management of suspected cases
- The postponement of mass gatherings is advised in areas with documented BVD transmission until transmission is interrupted. Event-specific risk assessments should be conducted using the WHO all-hazards mass gathering risk assessment tool. These assessments should be complemented by implementation of measures before, during, and after events, as outlined in WHO guidance for mass gatherings attended by individuals from areas with documented Bundibugyo virus detection, including the establishment of referral pathways and RT-PCR testing capacity.
- Event organisers should distribute participant information to all attendees before and during the event, covering BVD symptoms, what to do if feeling unwell, and available health services, in line with [WHO guidance for mass gathering events attended by individuals from areas with documented BDBV detection.](#)

Situation interpretation

The overall decline in reported cases and deaths is encouraging but does not yet indicate that the outbreak is under control. Transmission is becoming increasingly heterogeneous, with reductions in several established hotspots in Ituri occurring alongside expanding transmission in Nord-Kivu and Haut-Uélé and continued emergence of new affected health zones. Persistently high mortality, particularly community deaths, together with gaps in contact follow-up, referral and treatment capacity, infection prevention and control, and community acceptance, continue to create opportunities for transmission. The response should therefore become more anticipatory and geographically targeted, rapidly shifting surveillance, contact tracing, clinical care, IPC, community engagement, workforce and logistical capacity towards emerging and intensifying hotspots, while sustaining interventions in areas where transmission is declining to prevent resurgence.

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